

Resident: [REDACTED]
Pathologist: [REDACTED]

PATHOLOGIC DIAGNOSIS:

A. SPECIMEN DESIGNATED "OMENTUM, BIOPSY":

Fibroadipose tissue with fat necrosis, calcifications, and hemosiderin-laden macrophages.

B. SPECIMEN DESIGNATED "RIGHT ILEOCOLECTOMY" (34 cm):

MUCINOUS ADENOCARCINOMA, low grade, moderately differentiated (9.3 cm in greatest dimension).

Tumor is located in the cecum, forms a fungating mass, is ulcerative, and has an infiltrating border.

Tumor invades through the muscularis propria.

Proximal and distal resection margins are negative for tumor.

Invasive tumor is 7.9 cm from proximal resection margin, 23 cm from distal resection margin, and 2.5 cm from radial resection margin.

Lymphovascular invasion is not identified.

Extramural venous invasion is not identified.

Perineural invasion is not identified.

Peritumoral lymphoid response (including Crohn's-like infiltrate) is marked.

Residual adenoma is present.

Appendix with no significant pathologic change.

Regional lymph nodes (positive:total): 1:18.

AJCC Classification (6th edition): T3 N1 MX.

CLINICAL DATA:

History: [REDACTED] with colon CA and pulm. [REDACTED]

Operation: R colectomy.

Operative Findings: None specified.

Clinical Diagnosis: Colon CA.

TISSUE SUBMITTED:

A/1) Omental nodule.

B/2) R hemicolectomy --> Tissue Bank.

GROSS DESCRIPTION:

The specimen is received fresh in two parts, labeled with the patient's name and medical record number.

Part A, "#1. Omental nodule", consists of one white/tan calcified nodule (3.2 x 2.2 x 1.7 cm). The cut surface demonstrates calcified fat necrosis. The specimen is submitted for decalcification prior to histologic examination.

Micro A1-A5: Omental nodule, 1-2 frags each, [REDACTED]

Part B, "#2. Right hemicolectomy", consists of one ileocelectomy specimen including distal ileum (4.0 x 2.6 cm), a segment of colon (30.0 cm x 6.1 cm), and an appendix (7.2 x 0.8 cm) with a proximal stapled resection margin (3.1 cm, inked blue) and a distal stapled resection margin (6.5 cm, inked blue). A fungating tumor with central ulceration (9.3 x 5.1 x 0.9 cm) is present in the cecum abutting the ileocecal valve, 7.9 cm to the proximal stapled resection margin, and 23.0 cm to the distal stapled resection margin. A focal

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hemorrhagic lesion (2.5 x 1.8 x 0.4 cm) is present 15.0 cm distal to the mass, 8.0 cm to the distal stapled resection margin, and 24.0 cm to the proximal stapled resection margin. The tumor mass grossly invades into the pericolic fat. The remainder of the mucosa is unremarkable. Twenty one candidate lymph nodes are identified, the largest measuring 1.5 x 0.9 x 0.9 cm. Representative sections of the main mass and normal colon are submitted to the tissue bank. A representative section of the mass is submitted for GI stem cell culture. Gross photographs are taken.

Micro B1: Proximal resection margin, 2 frags, [REDACTED]
Micro B2: Distal resection margin, 1 frag, [REDACTED]
Micro B3: Tumor to normal mucosa, 1 frag, [REDACTED]
Micro B4: Tumor with deepest extent of invasion, 1 frag, [REDACTED]
Micro B5: Representative sections of tumor, 2 frags, [REDACTED]
Micro B6: Hemorrhagic lesion, 1 frag, [REDACTED]
Micro B7: Normal mucosa, 1 frag, [REDACTED]
Micro B8 through B9: Appendix, multi frags, [REDACTED]
Micro B10: Largest candidate lymph node bisected, 2 frags, [REDACTED]
Micro B11 through B17: Candidate lymph nodes, 3-4 frags each, [REDACTED]

CASE NUMBER: [REDACTED]

By his/her signature below, the senior physician certifies that he/she personally conducted a microscopic examination ("gross only" exam if so stated) of the described specimen(s) and rendered or confirmed the diagnosis(es) related thereto.

Final Diagnosis by [REDACTED], Electronically signed on [REDACTED]